Pertuzumab + trastuzumab maintenance

> **Confirm the individual supportive prescription.** This review aid does not generate a medication order. Use the current NCCP regimen, antiemetic guidance and local pharmacy plan; assess interactions, allergies and comorbidities.

Antiemetic and premedication review

- Minimal for HER2 antibodies; symptom-driven support
- Confirm the last treatment date with the team; long gaps can change the loading-dose plan.
- Echoes continue on maintenance. Report new breathlessness, ankle swelling or rapid weight gain.

Additional supportive care

- Confirm the last treatment date with the team; long gaps can change the loading-dose plan.
- Echoes continue on maintenance. Report new breathlessness, ankle swelling or rapid weight gain.
- Report diarrhoea early and follow the agreed hydration/support plan.
- Keep response-review appointments. Ask for support with fatigue, activity, sexual wellbeing and practical concerns.

Monitoring and handover

- Heart ultrasound: NCCP: baseline and every 12 weeks during HER2 treatment. NCCP baseline LVEF ≥50%; 4–8-week checks may be needed with impairment. ESC allows consideration of 6-monthly TTE after the first year in selected asymptomatic low/moderate-risk metastatic patients with normal assessments. This does not automatically override the NCCP 12-week schedule; document the agreed specialist/local plan.
- Response imaging: Usually every 2–4 months; earlier if clinically needed. Choose consistent imaging of disease sites. Brain MRI or other additional imaging is indication-specific, not a universal recurring item.
- Tumour markers, if useful: Only if included in the individual monitoring plan.
- Missed or delayed infusion: Whenever a treatment gap occurs. NCCP gap ≥6 weeks requires reloading of both antibodies. No routine percentage dose reduction for these antibodies.
- Separate early-breast pathways: At every transition. Do not label adjuvant trastuzumab/pertuzumab after TCHP as NCCP 00726. Use the relevant early-breast protocol.

Access, visits and patient preparation

- Port insertion and removal: The line team explains placement, dressing care and when normal activities can resume. A port is accessed with a special needle during treatment. Removal of the implanted device is a separate planned procedure when no longer needed; do not remove or adjust it yourself.
- PICC care and urgent line symptoms: A PICC usually needs regular dressing and flushing appointments even between treatments. Fever, redness, swelling, pain, leakage or damage requires prompt advice. Your hospital supplies the exact care instructions.
- Visit time and transport: First visits can take longer because of loading doses, premedicines and observation. Ask the unit about expected time, transport and any escort needed after sedation or line insertion.

Patient handover

Explain the current phase, how to take each medicine, which tests are expected next and who arranges and reviews them. Confirm urgent contact details. Provide the patient portal, visual guide, passport, treatment card and whole-treatment map.

Fever, shivers or suddenly feeling unwell: contact oncology immediately. Severe chest pain, breathing difficulty, collapse or seizure: emergency help, 112 / 999.

Sources

NCCP 00726 v3a · official regimen

ESC 2022 cardio-oncology guideline · cardiac surveillance and risk

NCCP supportive-care antiemetics for NCIS

SACTCheck v0.83.7 · Sources checked 2026-10-17 · Draft — independent clinical, pharmacy and patient/PPI review pending